PRD

CLAUDE CORPS · BASECAMP FELLOW TEMPLATE

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Host organization: Black Hills Works

1. Background

Tyler, our mentor named two priorities: embed in departments to build the AI workflows the one AI lead can't reach alone, and get the AT matching agent through accuracy testing and security hardening before production.

- BHWS's stated goal for AI: make information accessible to the person it's about, amplifying voices without replacing them.

The problem *(your problem statement, pasted word for word)*

- Adults with intellectual and developmental disabilities need AT(assistive technology) that fits how they communicate and live, but the request moves through slow handoffs between support staff, clinicians, and funders, so the right tools arrive late and the person has little visibility into where their request stands.

Who it's for

- Primary: DSPs, case managers, Finance department employees, who write notes and move AT requests forward.
- Secondary: adults with IDD that BHWS supports (about 600 in Rapid City), who need to see where their request stands.

Why it matters *(the consequence today, its size, and what's been tried)*

- The consequence today: people wait too long for tools that would help them communicate and live more independently, and they have little visibility into where the request is.
- Size: A large amount of time is wasted between employees, communicating ‘ping-pong’ emails back and forth.
- What's been tried: the AI lead built Claude skills in Billing and other departments, however usage varies greatly. The matching agent's build is complete, but no testing has been done yet.

Who decides

- Humans at each step. Support staff and case managers hand off the request. Clinicians (for example, an SLP or OT evaluation) and the funding review make the AT decision. The prototype decides nothing. Assumed owners; confirm with Tyler Wainwright in Week 1.

Constraints — answer all four; "ask [name] in Week 1" passes, a blank does not. Until the data-sensitivity person has agreed, the build runs on pod-written samples.

- **Data sensitivity** — what goes to Claude, and who agreed?
  For prototyping - synthetic only (five fictional adults in dummy_notes_200.csv) until approved.
- **IT and access** — who grants it, and how long?
  An Anthropic API key held in a local .env file and used only by the server. Ask Tyler in Week 1 who grants API access and for how long.
- **Staff-only or client-facing?**
  Staff-only for this prototype. The tracker is designed for the person later, but that view is not part of this build.
- **Approvals** — who says yes?

  Tyler Wainwright (Director of Information Security & AI) for data and access. Mary Duncan (CFO, executive sponsor) for spending. Ask Tyler in Week 1 who approves a staff demo.

2. Proposal

The solution, in a sentence or two

A prototype that turns a person's rough daily notes into a readable profile with an on-click summary, plus a visual AT handoff tracker, for support staff and case managers, so that AT requests are easier to understand and follow. It never decides what a person needs.

The idea we picked *(from Tuesday's brainstorm)*

- **Picked:** Combined the notes-to-summary idea and the handoff-tracker idea into one person's journey, using synthetic data.
- **Set aside, and why:** A stalled-request tracker alone (too thin without the summary); an in-my-own-words intake (needs the person present and picture-based design); a bounced-request checker (we don't have real funder checklists); a trial-feedback loop (no device trial data yet).
- **What would change our mind:** Staff say the notes already give them what they need,

or Week 1 shadowing shows the delay is in funding or equipment availability rather than handoffs.

User journey

Today

1. Staff write rough daily notes, and the signs of an AT need sit inside them (assumed).
2. Nobody connects those notes to an AT need, so the request starts late (assumed).
3. The request passes through handoffs: support staff, case manager, clinician evaluation, funding review (assumed order).
4. The person can't see where the request is and has to ask staff for updates (assumed).
5. The right tool arrives late, and in the meantime the person relies on a workaround.

With your solution

1. Staff notes are stored exactly as written.
2. Staff click Summarize. Claude Haiku returns a summary of only what the notes say, shown beside the raw notes.
3. The case manager reviews, edits, and hands off the request, which then appears in the tracker.
4. The person sees a plain-language view of where their request is.
5. Every decision about AT stays with a human. The summary is one input.

Crawl — your prototype

- **What goes in:** Five fictional adults' notes from dummy_notes_200.csv (200 rows: ID, name, date/time, note).
- **What comes out:** Pro file cards with raw notes in date order; an on-click Claude Haiku summary; an animated visual tracker through six placeholder stages.
- **What good looks like:** A staff reviewer can find every summary claim in the notes, sees no device or diagnosis advice, sees raw notes unchanged, and understands the tracker without explanation. The demo runs locally with one command.
- **What you'll build it with:** Node.js, Express, the Anthropic SDK (Claude Haiku), csv-parse, plain HTML, CSS and JavaScript.
- **The question this prototype answers:** Can a Claude summary of rough, messy notes be accurate enough that a sta ff member would use it to start an AT conversation?

Walk: A de-identified sample of real notes, approved by Tyler Wainwright and reviewed by real staff, with an edit-and-approve step before any handoff.

Run: A live handoff tracker tied to the case record, with a plain-language view for the person. Detailed profile setup, with client details stored in a secure db.

First test run *(how many of your checks the prototype passed — for example, 4 of 5 — and the worst failure you saw)*

- Not run yet.

Planned checks:

1) Every summary claim traces to a note.
2) No device or diagnosis language.
3) Raw notes unchanged after summarizing.
4) Summary runs only on click.
5) Tracker animates left to right.

Out of scope for now

- Device or service recommendations, and any approval or denial of AT.
- Automatic or background summaries.
- Real client data before approval.
- Connection to Therap or the EHR.
- Person-facing login, notifications, or saved tracker status.

3. Success

Outcome

- Staff can read a person's notes in one place and get a summary they trust enough to start an AT conversation. Longer term (the host's measure): an AT tool in production that touches the person directly within a year.

Early sign

- Staff reviewers rate summaries as matching the notes, and staff press Summarize without prompting.

Guardrail

- No summary suggests a diagnosis, device, or decision. No real data before approval. Raw notes are never edited.

Measurement plan

Measure	Definition	Source	Starting point	Target, and by when	Who checks, and how often
Summary traceability	Share of summary claims that match a note in the record	Spot check of 10 summaries against profile notes	Not measured yet	100% of claims traceable, by end of Crawl	Claude Corps pod, every test run
Guardrail violations	Summaries containing a diagnosis, device recommendation, or decision	Review of every summary in test runs	Not measured yet	Zero, by end of Crawl	Claude Corps pod, every test run; Tyler Wainwright reviews (proposed: every two weeks)
Staff usefulness	Staff reviewer rating (1-5) of: would this help me start an AT conversation?	Short survey after demo, 3+ staff reviewers	Not measured yet	Average 4 or higher, by end of Crawl	Claude Corps pod runs survey; results shared with Tyler Wainwright
Time to find AT-relevant notes	Minutes for a reviewer to locate and cite AT-related notes, with vs. without the prototype	Timed task on the same notes	Not measured yet	Measurable reduction; target set after baseline	Claude Corps pod, once per Walk

4. Open questions and risks

Questions

- Which AT request stages and owners exist in Therap or the EHR?
- Who approves use of real client notes with Claude?

- Who makes the final AT decision: case manager, clinician, or funder?
- **Next step:** Ask Tyler Wainwright in Week 1, and request one real AT request walked from first note to delivery.

Assumptions

- The main cause of late AT is handoff delay, not funding, equipment availability, or training.
- Staff would use the summary button instead of reading raw notes.
- The synthetic notes match the style and length of real notes.
- **Next step:** Shadow one real AT request and compare it against the timeline in User journey: Today.

Risks & mitigations

Risk	**Mitigation** ***(what you'll do about it — make it less likely, or less bad when it happens)***	**Next step**
Summary misses or distorts a note	Show raw notes beside the summary; staff spot checks; prompt limited to what the notes say	Add traceability spot check to test runs
Real client data sent to Claude before approval	Synthetic data only until Tyler signs off; document data handling	Ask Tyler in Week 1
Summary read as a recommendation or decision	Prompt forbids device and diagnosis language; label "AI summary: humans decide"; guardrail check	Add guardrail check to test list
Tracker mistaken for live data	Label it "visual mockup, not live data" on screen	Add label in build
Scope creep beyond three elements	Scope listed in CLAUDE.md; ask before adding features	Review scope at each check-in
API key exposed	Key only in .env, calls made server-side, .env in .gitignore	Check before sharing any folder

5. Cost and value

- **Cost:** Pod build time, and Claude Haiku API usage. One summary of about 40 notes is a small request, so expect pennies per demo run (verify against current pricing). No new software licenses.
- **Value:** Staff time saved reading notes for AT requests (to be measured); earlier identification of AT needs; a visible status for the person and case manager; a reusable method other providers can adopt.
- **Assumptions:** *(the spread across them is your realistic low and high)* Low: a summary saves about 5 minutes per AT case for one case manager. High: about 20 minutes per case across a team handling weekly AT cases. Both are guesses to test in Week 1.
- **How you'll check:** *(what you'll measure, the bar it has to clear, and when you'll decide to keep, change, or stop)* Measure summary accuracy and staff ratings on synthetic profiles with 3+ staff reviewers. Bar: zero unsupported claims and an average rating of 4 or higher. Keep if the bar clears; change the prompt or layout if accuracy misses; stop if staff say the summary adds nothing.

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